

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Co-amoxiclav 500/125mg tablets and Flucloxacillin 500mg capsules for the treatment of Minor Wound Care

Summary of NICE / NICE CKS Guidelines for Minor Wound Care

Assessment and Infection Control

- **Minor wounds:** Minor cuts, abrasions, puncture wounds, or bite wounds requiring assessment for infection risk.
- **Infection assessment:** Look for signs of infection: increased pain, erythema, warmth, swelling, purulent discharge, lymphangitis.
- **High-risk wounds:** Animal bites, human bites, contaminated/dirty wounds, puncture wounds, deep wounds.

When Antibiotics Are Indicated

- **Infected wounds:** Signs of active infection warrant antibiotic treatment.
- **High-risk clean wounds:** Consider prophylactic antibiotics for animal/human bites and contaminated wounds.
- **Tetanus status:** Check tetanus immunisation status. Administer booster if needed (separate from this PGD).

Co-amoxiclav Use

- **Indication:** Infected wounds, particularly those contaminated with mouth flora (bites) or environmental bacteria.
- **Dosing:** 500/125mg three times daily for 5-7 days.

Flucloxacillin Use

- **Indication:** Cellulitis and infected wounds, particularly for Staphylococcus aureus coverage.
- **Dosing:** 500mg four times daily for 5-7 days.

Patient Group Direction

for the supply/administration of

Co-amoxiclav 500/125mg tablets

for the treatment of

Minor Wound Care

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and
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	requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Treatment of infected wounds and animal/human bites
Inclusion criteria	Adults aged 18 years and over Infected wounds with signs of bacterial infection (erythema, swelling, purulent discharge) Animal bites or human bites with signs of infection or high infection risk Contaminated wounds where antibiotics are indicated Capable of giving valid informed consent
Exclusion criteria	Known penicillin or cephalosporin allergy History of co-amoxiclav-induced jaundice or hepatic dysfunction Acute lymphoblastic leukaemia or cytomegalovirus infection (theoretical risk due to amoxicillin toxicity) Infectious mononucleosis or acute leukemia (theoretical increased risk of rash) Severe hepatic dysfunction (avoid use) Severe renal impairment (eGFR <30 mL/min/1.73m²)
Cautions	Mild to moderate renal impairment: dose adjustment may be needed Mild to moderate hepatic impairment: use with caution

	Pregnancy and breastfeeding: generally safe but ensure consent Drug interactions: may reduce efficacy of oral contraceptives; advise additional contraception Anticoagulant interactions: possible increased anticoagulant effect
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Co-amoxiclav 500/125mg tablets
Legal category	POM
Storage	Store below 25°C. Protect from moisture. Keep in original container.
Route/method of administration	Oral
Dose and frequency	500/125mg (one tablet) three times daily To be taken with or without food, though food may reduce GI upset
Quantity to be administered and/or supplied	21-28 tablets (7-10 day course)
Maximum or minimum treatment period	5-7 days
Adverse effects	Very common: diarrhoea and nausea Common: rash, vomiting, vaginitis/candidiasis Uncommon: abdominal pain, indigestion, headache Rare: allergic reactions (urticaria, Stevens-Johnson syndrome, TEN), hepatitis, cholestasis, C. difficile infection, agranulocytosis

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment

	• details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)
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Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	Complete the full course of antibiotics, even if the wound appears to be healing well. Take each dose at regular intervals throughout the day. If using oral contraception, use additional contraceptive methods during the course and for 7 days after stopping. Maintain good wound hygiene: keep the wound clean and covered as appropriate. Monitor for signs of worsening infection: increased redness, swelling, warmth, pus, or spreading erythema. Seek medical advice immediately if signs of infection worsen, fever develops, or systemic symptoms appear (malaise, chills). Report any allergic reactions (rash, swelling of lips/throat, breathing difficulties) immediately. Seek urgent medical attention if experiencing severe diarrhoea or signs of <i>C. difficile</i> infection. Ensure tetanus status is reviewed; additional tetanus prophylaxis may be required separately.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance: Wounds: assessment and management
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
1/11/25	31/10/26		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Patient Group Direction

for the supply/administration of

Flucloxacillin 500mg capsules

for the treatment of

Minor Wound Care

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Treatment of skin and soft tissue infections, cellulitis, and infected wounds
Inclusion criteria	Adults aged 18 years and over Cellulitis or infected wounds with signs of bacterial infection Wound infections caused or suspected to be caused by <i>Staphylococcus aureus</i> Skin and soft tissue infections requiring antibiotic therapy Capable of giving valid informed consent
Exclusion criteria	Known penicillin or beta-lactam allergy History of flucloxacillin-induced hepatitis or jaundice

	History of hepatic dysfunction with flucloxacillin exposure Severe hepatic dysfunction
Cautions	Hepatic impairment: avoid if history of hepatic dysfunction; use with caution in mild-moderate impairment Renal impairment: generally safe but monitor for accumulation Pregnancy and breastfeeding: generally safe but ensure consent Drug interactions: may reduce efficacy of oral contraceptives; advise additional contraception
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Flucloxacillin 500mg capsules
Legal category	POM
Storage	Store below 25°C. Keep in original container. Protect from moisture.
Route/method of administration	Oral
Dose and frequency	500mg (one capsule) four times daily To be taken on an empty stomach (1 hour before or 2 hours after food) for optimal absorption
Quantity to be administered and/or supplied	20-28 capsules (5-7 day course)
Maximum or minimum treatment period	5-7 days
Adverse effects	Very common: nausea, diarrhoea Common: vomiting, abdominal discomfort, rash, vaginitis Uncommon: abdominal pain, indigestion Rare: hypersensitivity reactions (urticaria, anaphylaxis), hepatitis, cholestasis, C. difficile infection, jaundice

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: that valid informed consent was given

	• name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)
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